

Patient NAME : Mr Utkarsh Dubey
 DOB/Age/Gender : 26 Y/Male
 Patient ID / UHID : 14135499/RCL13812679
 Referred BY : Self
 Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report
 Barcode NO : 28974708
 Sample Type : Whole blood EDTA
 Report Date : Oct 06, 2025, 01:47 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Medline Male Package

Complete Blood Count (CBC)

RBC Parameters			
Hemoglobin <i>Cyanide free colorimetric</i>	15.1	g/dL	13.0 - 17.0
RBC Count <i>Electrical impedance</i>	5.2	10 ⁶ /μl	4.5 - 5.5
PCV <i>Calculated</i>	46.4	%	40 - 50
MCV <i>Calculated</i>	88.7	fl	83 - 101
MCH <i>Calculated</i>	28.8	pg	27 - 32
MCHC <i>Calculated</i>	32.5	g/dL	31.5 - 34.5
RDW (CV) * <i>Calculated</i>	13.7	%	11.6 - 14.0
RDW-SD * <i>Calculated</i>	45.8	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	5.8	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils <i>Laser based Flow-cytometry</i>	55.9	%	40-80
Lymphocytes <i>Laser based Flow-cytometry</i>	31.7	%	20-40
Monocytes <i>Laser based Flow-cytometry</i>	7.1	%	2-10
Eosinophils <i>Laser based Flow-cytometry</i>	5.1	%	1-6
Basophils <i>Laser based Flow-cytometry</i>	0.2	%	<2
Absolute Leukocyte Counts *			
Neutrophils. * <i>Calculated</i>	3.24	10 ³ /μl	2 - 7
Lymphocytes. * <i>Calculated</i>	1.84	10 ³ /μl	1 - 3
Monocytes. * <i>Calculated</i>	0.41	10 ³ /μl	0.2 - 1.0
Eosinophils. * <i>Calculated</i>	0.3	10 ³ /μl	0.02 - 0.5

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
 MD (Pathology)
 Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉️ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey
 DOB/Age/Gender : 26 Y/Male
 Patient ID / UHID : 14135499/RCL13812679
 Referred BY : Self
 Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report
 Barcode NO : 28974708
 Sample Type : Whole blood EDTA
 Report Date : Oct 06, 2025, 01:47 PM.



Test Description	Value(s)	Unit(s)	Reference Range
Basophils. * <i>Calculated</i>	0.01	10 ³ /μl	0.02 - 0.5
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	269	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) * <i>Calculated</i>	6.9	fL	9.3 - 12.1
PCT * <i>Calculated</i>	0.2	%	0.17 - 0.32
P-LCR * <i>Calculated</i>	16.4	%	18 - 50
P-LCC * <i>Calculated</i>	44	10 ⁹ /L	44 - 140
Mentzer Index * <i>Calculated</i>	17.06	%	> 13

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
 MD (Pathology)
 Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉️ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM.

Report STATUS : Final Report

Barcode NO : 28974708

Sample Type : Whole blood EDTA

Report Date : Oct 06, 2025, 03:04 PM.

Peripheral Smear General Blood Picture

RBCs	Predominantly normocytic normochromic.
WBCs	Normal in number and distribution.
Platelets	Adequate in number on smear.
Hemoparasite	No Hemoparasite seen.
Impression	Normocytic Normochromic blood picture.

Advise : Kindly correlate clinically.

A peripheral blood smear (also known as a general blood picture or blood film) is a laboratory test that involves examining a drop of blood under a microscope to evaluate the cells present. This test provides detailed information about the size, shape, and appearance of blood cells, which can help diagnose a variety of blood disorders and conditions.

Clinical Use:

1. Diagnosing Anaemias: Helps determine the type and cause of anaemia.
2. Identifying Infections: Detects the presence of abnormal or immature white blood cells.
3. Blood Disorders: Diagnoses conditions like leukaemia, lymphoma, and other blood cancers.
4. Monitoring Treatment: Evaluates the effectiveness of treatment for blood disorders and infections.

Interpretation:

1. Normal Blood Smear: Shows normal size, shape, and colour of red blood cells, normal types and appearance of white blood cells, and normal number and appearance of platelets.
2. Abnormal Blood Smear: Can indicate a variety of conditions, depending on the specific abnormalities observed.

A peripheral blood smear is a valuable diagnostic tool that provides detailed information about the health and characteristics of blood cells, assisting in the diagnosis and management of a wide range of medical conditions.

Interpretation:

A peripheral blood smear (also known as a general blood picture or blood film) is a laboratory test that involves examining a drop of blood under a microscope to evaluate the cells present. This test provides detailed information about the size, shape, and appearance of blood cells, which can help diagnose a variety of blood disorders and conditions.

Clinical Use:

1. Diagnosing Anaemias: Helps determine the type and cause of anaemia.
2. Identifying Infections: Detects the presence of abnormal or immature white blood cells.
3. Blood Disorders: Diagnoses conditions like leukaemia, lymphoma, and other blood cancers.
4. Monitoring Treatment: Evaluates the effectiveness of treatment for blood disorders and infections.

Interpretation:

1. Normal Blood Smear: Shows normal size, shape, and colour of red blood cells, normal types and appearance of white blood cells, and normal number and appearance of platelets.
2. Abnormal Blood Smear: Can indicate a variety of conditions, depending on the specific abnormalities observed.

A peripheral blood smear is a valuable diagnostic tool that provides detailed information about the health and characteristics of blood cells, assisting in the diagnosis and management of a wide range of medical conditions.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28974708

Sample Type : Whole blood EDTA

Report Date : Oct 06, 2025, 02:29 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate MODIFIED WESTERGREN	9	mm/hr	0 - 10
---	---	-------	--------

Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and Lewis practical hematology

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28974708

Sample Type : Whole blood EDTA

Report Date : Oct 06, 2025, 02:56 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) HPLC	5.4	%	<5.7
Estimated Average Glucose * calculated.	108.28		

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled. 2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609

 ccsupport@redcliffelabs.com

 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28501373

Sample Type : FLUORIDE F

Report Date : Oct 06, 2025, 02:24 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Glucose Fasting

Glucose Fasting <i>Hexokinase</i>	87	mg/dL	70 - 100
--------------------------------------	----	-------	----------

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association**Comment :**

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy, insulinoma, or various liver diseases.

Note

1. The diagnosis of Diabetes requires a fasting plasma glucose of $\geq$ 126 mg/dL or a random / 2 hour plasma glucose value of $\geq$ 200 mg/dL with symptoms of diabetes mellitus.
2. Very high glucose levels ($>$ 450 mg/dL in adults) may result in Diabetic Ketoacidosis.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Dayanand J Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

928-909-0609

ccsupport@redcliffelabs.com

www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey
 DOB/Age/Gender : 26 Y/Male
 Patient ID / UHID : 14135499/RCL13812679
 Referred BY : Self
 Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report
 Barcode NO : 28924557
 Sample Type : Serum
 Report Date : Oct 06, 2025, 02:53 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Liver Function Test (LFT)

Bilirubin Total <i>Photometric</i>	1.5	mg/dL	0.2 - 1.2
Bilirubin Direct <i>Diazo Reaction</i>	0.4	mg/dL	0.0 - 0.5
Bilirubin Indirect * <i>Calculation (T Bil - D Bil)</i>	1.1	mg/dL	0.1 - 1.0
SGOT/AST <i>IFCC without P5P</i>	28	U/L	5 - 34
SGPT/ALT <i>IFCC without P5P</i>	19	U/L	0 to 55
SGOT/SGPT Ratio *	1.47	-	-
Alkaline Phosphatase <i>IFCC</i>	71	U/L	40 - 150
Total Protein <i>Biuret</i>	6.7	g/dL	6.4 - 8.3
Albumin <i>BCG</i>	4.43	gm/dL	3.8 - 5.0
Globulin * <i>Calculation (T.P - Albumin)</i>	2.27	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * <i>Calculation (Albumin/Globulin)</i>	1.95	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * <i>Photometric</i>	20	U/L	12 - 64

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

1. **AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
2. **ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
3. **Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

1. **Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
2. **High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
3. **Low albumin:** Caused by poor diet, kidney, or liver disease.
4. **High albumin:** Usually due to severe dehydration.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:53 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Lipid Profile

Total Cholesterol <i>Enzymatic - Cholesterol Oxidase</i>	193	mg/dL	<200
Triglycerides <i>Colorimetric - Lip/Glycerol Kinase</i>	90	mg/dL	<150
HDL Cholesterol <i>Accelerator Selective Detergent</i>	44	mg/dL	>40
Non HDL Cholesterol * <i>Calculated</i>	149	mg/dL	<130
LDL Cholesterol <i>Calculated</i>	131	mg/dL	<100
V.L.D.L Cholesterol * <i>Calculated</i>	18	mg/dL	< 30
Chol/HDL Ratio * <i>Calculated</i>	4.39	Ratio	3.5 - 5.0
HDL/ LDL Ratio * <i>Calculated</i>	0.34	Ratio	0.5 - 3.0
LDL/HDL Ratio * <i>Calculated</i>	2.98	Ratio	-

Interpretation:

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609

 ccsupport@redcliffelabs.com

 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:53 PM.



Test Description	Value(s)	Unit(s)	Reference Range
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque		
Moderate Risk	2 major ASCVD risk factors		
Low Risk	0-1 major ASCVD risk factors		
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors			
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use		
2. Family history of premature ASCVD	4. High blood pressure		
5. Low HDL			

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:53 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Iron Studies

Iron <i>Ferene</i>	101	µg/dL	65 - 175
TIBC,(Total Iron Binding Capacity) <i>Calculated</i>	321	µg/dL	250 - 450
UIBC <i>Ferene</i>	220	µg/dL	69 - 240
Transferrin Saturation <i>Method :Derived from IRON and TIBC values</i>	31.46	%	-

Interpretation:

Increased levels due to iron ingestion or ineffective erythropoiesis. Decreased levels due to infection, inflammation, malignancy, menstruation and Fe deficiency. Needs to be taken into consideration with TIBC. Transferrin Saturation:- Low level Transferrin Saturation can indicate iron deficiency, erythropoiesis, infection, or inflammation. High level Transferrin Saturation can indicate recent ingestion of dietary iron, ineffective erythropoiesis, haemochromatosis or liver disease. High TIBC, UIBC, or transferrin usually indicates iron deficiency, but they are also increased in pregnancy and with the use of oral contraceptives. Low TIBC, UIBC, or transferrin may occur if someone has: Hemochromatosis, Certain types of anemia due to accumulated iron, Malnutrition, kidney disease that causes a loss of protein in urine.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033


928-909-0609

ccsupport@redcliffelabs.com

www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 03:01 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Vitamin B12 / Cyanocobalamin

Vitamin - B12 CMIA	156	pg/mL	187 - 883
-----------------------	-----	-------	-----------

Interpretation:

Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.

Causes of vitamin B12 deficiency include: Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉️ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:59 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy CMIA	20.2	ng/mL	Deficient <20 Insufficient 21 - 29 Sufficient 30 - 100
--------------------------------	------	-------	--

Interpretation:

25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:59 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Thyroid Profile Total

Triiodothyronine (T3) CMIA	116.2	ng/dL	35 - 193
Total Thyroxine (T4) CMIA	7.2	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) CMIA	1.46	mIU/L	0.35 - 4.94

Interpretation:

Pregnancy	Reference Range TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Clinical Use:

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance**Decreased TSH:** Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. **Primary Hypothyroidism:** High TSH levels.
2. **Secondary/Tertiary Hypothyroidism:** Low TSH levels.
3. **Euthyroid Sick Syndrome:** Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Normal	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



 Dr. Dayanand J. Sonkawade
 MD (Pathology)
 Consultant Pathologist.


Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033


928-909-0609

ccsupport@redcliffelabs.com

www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:53 PM.

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Prostate Specific Antigen (PSA) Total

Prostate Specific Antigen-Total (PSA-Total) CMIA	1	ng/mL	<4.0
---	---	-------	------

Interpretation:

- 1· Prostate specific antigen (PSA), a member of the human kallikrein gene family, is a serine protease with chymotrypsin-like activity.
- 2· The major site of PSA production is the glandular epithelium of the prostate. PSA has also been found in breast cancers, salivary gland neoplasms, periurethral and anal glands, cells of the male urethra, breast milk, blood and urine.
- 3· The combined use of DRE (digital rectal examination) and PSA has been shown to result in an increased detection of early stage prostate cancer.
- 4· PSA testing can have significant value in detecting metastatic or persistent disease in patients following surgical or medical treatment of prostate cancer.
- 5· Persistent elevation of PSA following treatment, or an increase in a post-treatment PSA level is indicative of recurrent or residual disease. PSA testing is widely accepted as an adjunctive test in the management of prostate cancer patients.

Increased Levels

Prostate cancer
Benign Prostatic Hyperplasia
Prostatitis
Genitourinary infections

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.



Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey
DOB/Age/Gender : 26 Y/Male
Patient ID / UHID : 14135499/RCL13812679
Referred BY : Self
Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report
Barcode NO : 28924557
Sample Type : Serum
Report Date : Oct 06, 2025, 02:53 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

KFT Basic

Blood Urea <i>Urease</i>	20	mg/dL	19 - 44.1
Bun * <i>Calculated</i>	9.35	mg/dL	8.9 - 20.6
Creatinine <i>Photometric</i>	0.88	mg/dL	0.72 - 1.25
eGFR (CKD-EPI) *	121.61	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * <i>Calculated</i>	10.63		12 - 20
Urea / Creatinine Ratio * <i>Calculated</i>	22.73		25.68- 42.8
Uric Acid <i>Uricase</i>	5	mg/dL	3.5 - 7.2
Calcium Serum <i>Arsenazo III</i>	9.4	mg/dL	8.4 - 10.2

Interpretation:

A kidney function test, is a critical diagnostic tool that assesses the health and efficiency of the kidneys through a series of blood tests. These tests commonly include urea, creatinine, blood urea nitrogen (BUN), calcium, and uric acid levels.

Urea: Urea is a waste product formed from the breakdown of proteins. Elevated levels may indicate impaired kidney function or other conditions affecting protein metabolism.

Creatinine: Creatinine is a waste product from muscle metabolism. Elevated levels in the blood can indicate reduced kidney function because healthy kidneys filter creatinine from the blood into the urine.

Blood Urea Nitrogen (BUN): BUN measures the amount of nitrogen in the blood that comes from urea. Elevated levels can indicate kidney dysfunction, dehydration, or other conditions affecting the kidneys or liver.

Calcium: Calcium levels are essential for bone health and other bodily functions. Abnormal calcium levels can sometimes indicate kidney disease, especially when calcium levels are too high (hypercalcemia).

Uric Acid: Uric acid is a waste product from the breakdown of purines, which are found in certain foods and are also formed by the body. Elevated uric acid levels can lead to conditions like gout or kidney stones.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.



Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:54 AM

Report STATUS : Final Report

Barcode NO : 28924557

Sample Type : Serum

Report Date : Oct 06, 2025, 02:53 PM.



Test Description	Value(s)	Unit(s)	Reference Range
Factors such as age, gender, race, muscle mass, and medications can influence test results, necessitating careful interpretation by healthcare providers. Regular kidney function testing is crucial for detecting and monitoring kidney conditions early, allowing for timely intervention and management to preserve kidney health and prevent complications associated with kidney disease.			
"eGFR test is applicable for patients aged 18 years or more."			

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.

Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:45 AM

Report STATUS : Final Report

Barcode NO : 29020660

Sample Type : Spot Urine

Report Date : Oct 06, 2025, 02:59 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Urine Routine and Microscopic Examination

Physical Examination *			
Volume *	20	mL	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Clear	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination *			
Reaction (pH) <i>Double Indicator</i>	5	-	4.5 - 8.0
Specific Gravity <i>Ion Exchange</i>	1.025	-	1.010 - 1.030
Urine Glucose (sugar) <i>Oxidase / Peroxidase</i>	Negative	-	Negative
Urine Protein (Albumin) <i>Acid / Base Colour Exchahnge</i>	Negative	-	Negative
Urine Ketones (Acetone) <i>Legals Test</i>	Negative	-	Negative
Blood <i>Peroxidase Hemoglobin</i>	Negative	-	Negative
Leucocyte esterase <i>Enzymatic Reaction</i>	Negative	-	Negative
Bilirubin Urine <i>Coupling Reaction</i>	Negative	-	Negative
Nitrite <i>Griless Test</i>	Negative	-	Negative
Urobilinogen * <i>Ehrlichs Test</i>	Normal	-	Normal
Microscopic Examination *			
Pus Cells (WBCs) *	1-2	/hpf	0 - 5
Epithelial Cells *	1-2	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Absent	-	Absent
Protozoa *	Absent	-	Absent

Interpretation:**URINALYSIS-** Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.**Protein:** Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.


Dr. Dayanand J. Sonkawade
MD (Pathology)
Consultant Pathologist.

Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

📞 928-909-0609

✉ ccsupport@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Utkarsh Dubey

DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 14135499/RCL13812679

Referred BY : Self

Sample Collected : Oct 06, 2025, 08:45 AM

Report STATUS : Final Report

Barcode NO : 29020660

Sample Type : Spot Urine

Report Date : Oct 06, 2025, 02:59 PM.



Test Description	Value(s)	Unit(s)	Reference Range
Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.			
Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.			
Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.			
Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.			
Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.			
pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.			
Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.			
Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.			
Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.			

*** End Of Report ***

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Dayanand J Sonkawade
MD (Pathology)
Consultant Pathologist.

Booking Centre :- BAJAJ_FINSERV NOIDA, .

Processing Lab :- Survey No.185, CTS No-4269, Prem Sagar, Near PCMC AUDM Bakinghall, PimpriChinchwad, Pune, Maharashtra, 411033

 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Bio-Rad CDM System
VII Inst. #1. SN--16213

PATIENT REPORT
V2TURBO_A1c_2.0

Patient Data

Sample ID: 28974708
Patient ID:
Name:
Physician:
Sex:
DOB:

Analysis Data

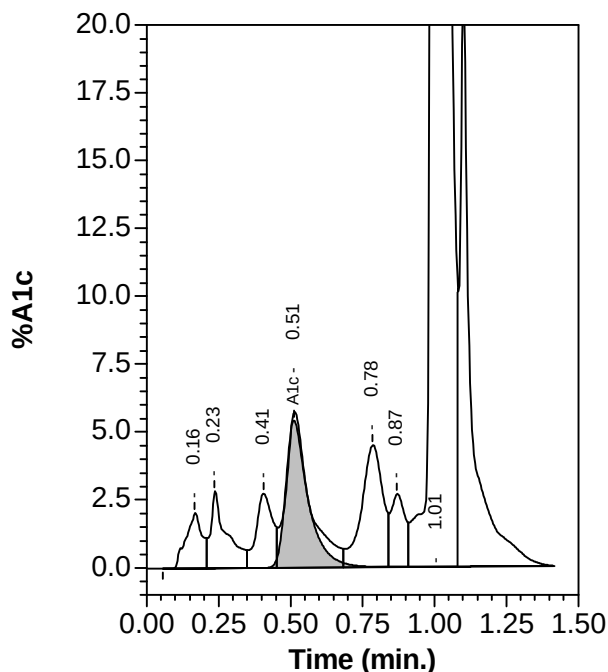
Analysis Performed: 10/06/2025 14:40:34
Injection Number: 10021
Run Number: 234
Rack ID: 0006
Tube Number: 2
Report Generated: 10/06/2025 14:52:45
Operator ID:

Comments:

Peak Name	NGSP %	Area %	Retention Time (min)	Peak Area
A1a	---	1.2	0.165	12512
A1b	---	1.7	0.234	17342
LA1c	---	1.6	0.406	16727
A1c	5.4	---	0.511	46406
P3	---	3.4	0.783	35623
P4	---	1.4	0.868	14261
Ao	---	86.4	1.006	907144

Total Area: 1,050,015

HbA1c (NGSP) = 5.4 %



Terms and Conditions of Reporting

1. The presented findings in the Reports are intended solely for informational and interpretational purposes by the referring physician or other qualified medical professionals possessing a comprehensive understanding of reporting units, reference ranges, and technological limitations. The laboratory shall not be held liable for any interpretation or misinterpretation of the results, nor for any consequential or incidental damages arising from such interpretation.
2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.
3. It is to be noted that variations in results may occur between different laboratories and over time, even for the same parameter for the same Customer. The assays are performed and conducted in accordance with standard procedures, and the reported outcomes are contingent on the specific individual assay methods and equipment(s) used, as well as the quality of the received specimen.
4. This report shall not be deemed valid or admissible for any medico-legal purposes.
5. The Customers assume full responsibility for apprising the Company of any factors that may impact the test finding. These factors, among others, includes dietary intake, alcohol, or medication / drug(s) consumption, or fasting. This list of factors is only representative and not exhaustive.

HOW WAS YOUR EXPERIENCE?

We value your feedback, please scan with your phone camera
and rate overall experience.

How likely are you to recommend Redcliffe Labs to your family & friends



Not Likely

 Very Likely



Scan with your phone camera